# Hypertension / Suspected Hypertensive Emergency

**AM-04 | Adult Medical**

## Quick flow

**01 Confirm Blood Pressure [EMT / AEMT / Paramedic]**

Correct cuff size • repeat an extreme or inconsistent value manually when feasible

**02 End-Organ Emergency? [EMT / AEMT / Paramedic]**

Stroke/CVA • ACS • pulmonary edema • aortic syndrome • encephalopathy • pregnancy

**03 Undifferentiated CVA? [Paramedic]**

If SBP >220 or DBP >110: labetalol 10 mg slow IV over 2 min • reassess • repeat 10 mg once after at least 10 min only if still above target

**04 Stroke BP Target [Paramedic]**

Goal SBP ≤220 and DBP ≤110 • do not lower further without a condition-specific order

**05 Other Acute Injury [EMT / AEMT / Paramedic / Medical Control]**

Open the linked ACS, pulmonary-edema, aortic, obstetric, or facility-transfer pathway • Medical Control for non-stroke antihypertensive treatment

**06 No Acute Injury [EMT / AEMT / Paramedic]**

Address pain, anxiety, hypoxia, urinary retention, stimulant exposure, and missed medications • do not give an antihypertensive solely to normalize BP

**07 Transport + Reassess [EMT / AEMT / Paramedic]**

Serial BP • HR/rhythm • neurologic status • chest symptoms • perfusion

## Overview

- Treat acute end-organ injury, not an isolated blood-pressure number.
- Rapid, non-targeted blood-pressure reduction can reduce cerebral, coronary, and renal perfusion.

## Provider Actions

Confirm the measurement, identify the condition causing the elevation, and avoid indiscriminate blood-pressure reduction.

**EMT**

- Repeat BP when feasible, perform a stroke screen, and assess chest or back pain, dyspnea, pregnancy-related symptoms, seizure, altered mental status, visual symptoms, and perfusion.
- Do not administer a patient's home antihypertensive unless specifically authorized.

**AEMT**

- Perform all EMT actions; establish IV access when clinically indicated without delaying transport, continue monitoring, and provide condition-specific care.

**Paramedic**

- Perform all prior care and obtain a 12-lead ECG when it will not delay time-critical care or transport.
- For undifferentiated suspected CVA with manually confirmed SBP above 220 mm Hg or DBP above 110 mm Hg, give labetalol 10 mg slow IV over 2 minutes. Reassess every 5 minutes; if still above the target after at least 10 minutes, repeat 10 mg once. Maximum total dose is 20 mg. The target is SBP 220 mm Hg or less and DBP 110 mm Hg or less.

- For a non-stroke hypertensive emergency, use the linked condition-specific pathway. Any labetalol use outside the undifferentiated-CVA branch requires Medical Control to define the indication, dose, and endpoint.

## Indications

- Markedly elevated BP with symptoms or findings concerning for acute end-organ injury.
- Undifferentiated suspected CVA with manually confirmed SBP above 220 mm Hg or DBP above 110 mm Hg.

## Contraindications

- Do not lower BP solely to normalize an isolated number without a protocol-defined indication.
- Do not use the undifferentiated-CVA labetalol branch for post-TNK transfer; follow AM-06 and written facility orders.
- Do not give labetalol for pulmonary edema, acute decompensated heart failure, cardiogenic shock, bradycardia, advanced heart block, or active bronchospasm.

## Assessment | End-organ screen

- New focal neurologic deficit, altered mental status, seizure, chest or back pain, pulmonary edema, visual symptoms, pregnancy-related symptoms, or reduced perfusion.
- Document antihypertensive adherence, stimulant or sympathomimetic exposure, baseline neurologic status, symptom onset, and last known well when CVA is suspected.
- For suspected CVA, obtain and document serial BP, HR, rhythm, glucose, focused neurologic findings, and response to any treatment.

## Treatment steps

- Verify the reading and address pain, anxiety, hypoxia, urinary retention, stimulant exposure, and the underlying emergency.
- Use UP-14 for suspected stroke, AC-04 for ACS, AC-05 for pulmonary edema, the aortic pathway or Medical Control for suspected aortic syndrome, and AO-03 for pregnancy-related hypertension.
- For undifferentiated suspected CVA with manually confirmed SBP above 220 mm Hg or DBP above 110 mm Hg, give labetalol 10 mg slow IV over 2 minutes. Reassess BP, HR, rhythm, perfusion, and neurologic status every 5 minutes.
- If still above target after at least 10 minutes, repeat labetalol 10 mg slow IV once. Maximum total dose is 20 mg. The target is SBP 220 mm Hg or less and DBP 110 mm Hg or less; do not lower further without a condition-specific order.
- For a non-stroke confirmed hypertensive emergency, contact Medical Control for any antihypertensive treatment not explicitly authorized by a linked condition-specific pathway.
- Transport symptomatic patients and reassess frequently; do not let BP treatment delay stroke alert, definitive imaging, or time-critical transport.

## Medications

### Labetalol

Undifferentiated suspected CVA with SBP >220 or DBP >110: 10 mg slow IV over 2 min; may repeat 10 mg once after at least 10 min if still above target; maximum total 20 mg

- Paramedic standing order only for the defined undifferentiated-CVA branch after manual confirmation.
- Goal BP: SBP ≤220 and DBP ≤110. Do not use this target for possible thrombolysis/TNK or post-TNK transfer.
- For any other hypertensive emergency, Medical Control must define the indication, dose, and endpoint.

## Warnings / precautions

- Rapid uncontrolled BP reduction can worsen cerebral, coronary, or renal perfusion.
- Do not administer labetalol to a patient with pulmonary edema, acute decompensated heart failure, cardiogenic shock, bradycardia, advanced heart block, or active bronchospasm.
- Do not use the undifferentiated-CVA target after thrombolysis; AM-06 and written facility orders govern post-TNK treatment.

## Clinical pearls

- Severe hypertension without acute end-organ injury is not usually a prehospital medication emergency.
- Accurate cuff size and manual confirmation prevent major treatment errors.
- A suspected stroke is undifferentiated until imaging; do not delay destination or imaging for BP normalization.

## Related pathways / contacts

**UP-14 Suspected Stroke** - Open for time-critical stroke assessment, alert, destination, and the matching undifferentiated-CVA BP branch.

**AM-06 TNK Transfer** - Open for post-TNK transfer; the tighter BP target and written facility orders govern.

**AC-04 Acute Coronary Syndrome / STEMI** - Open for ischemic chest discomfort or ECG concern.

**AC-05 CHF / Acute Pulmonary Edema** - Open for pulmonary edema or acute decompensated heart failure; do not give labetalol under AM-04.

**AO-03 OB/GYN Emergency** - Open for pregnancy-related hypertension or eclampsia concern.

### Source and references

American Heart Association/American Stroke Association. 2026 Guideline for the Early Management of Patients With Acute Ischemic Stroke. | Tennessee EMS ALS/BLS Blended Protocol Guidelines. | Claiborne County EMS medical-director decision, October 16, 2026.

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